Joseph Medical Clinic

59212 Weber Prairie
Baytown, TX
Phone 346-555-9847

Itemized Medical Bill

Patient:	Destiny Smith
Date of birth:	1961-09-13
Insurance claim:	C-1489
Date of injury:	2024-12-26
First visit:	2024-12-30
Last visit:	2025-06-25
Number of visits:	44

Services

Service	Units	Charge
99203 New patient evaluation	1	$253.07
98941 Spinal manipulation	6	$758.04
97110 Therapeutic exercise	8	$1,282.57
97124 Massage therapy	9	$805.49
72040 X-ray, cervical spine	11	$9,334.80
20552 Trigger point injection	9	$4,966.23
TOTAL		$17,400.20

Recent visits

Date	Service	Charge
2025-05-31	97124 Massage therapy	$111.90
2025-06-04	98941 Spinal manipulation	$128.66
2025-06-08	20552 Trigger point injection	$696.97
2025-06-12	97110 Therapeutic exercise	$128.86
2025-06-22	97110 Therapeutic exercise	$157.99
2025-06-25	20552 Trigger point injection	$680.99
2025-06-25	97112 Neuromuscular re-education	$108.24

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Treatment History and Summary

Patient:	Destiny Smith
Treating provider:	Charles Hensley, DC
Diagnosis:	concussion

Treatment history by month

Month and services	Visits	Charges
December 2024: evaluation, imaging	2	$1,088.39
January 2025: chiropractic adjustment, imaging, massage therapy, pain management, physical therapy	16	$6,863.78
February 2025: chiropractic adjustment, imaging, pain management	5	$2,459.70
March 2025: chiropractic adjustment, massage therapy, physical therapy	4	$484.40
April 2025: imaging, massage therapy, pain management	5	$2,074.41
May 2025: imaging, massage therapy, physical therapy	7	$2,636.05
June 2025: chiropractic adjustment, pain management, physical therapy	5	$1,793.47

Treatment summary

Patient presented following a collision sustained while the vehicle was turning left, when the patient's vehicle was struck on the dirver side. Presenting complaints inculded headache, dizziness, and difficulty concentrating, consistent with a concussion. The mechanism and force of the impact are considered well matched to the reported injury. Over the course of 44 visits, care followed a typical progression, including symptom monitoring, vestibular and cognitive therapy, and graduated return-to-activity guidance. Visits became less frequent as symptoms improved and the patient was discharged to self-directed home exercises. Several examination findings, including balance and oculomotor measures, support the diagnosis, while other neurological and imaging assessments returned within normal limits. No prior injury to the head or neck has been identified in the available history. Two similar claim charges submitted on the same date are under separate review and are not reflected in this clinical assessment.